

Neurological

N-05 Syncope / Near Syncope BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Assess level of consciousness
- Evaluation of:
 - Cardiac rhythm, precipitating factors
 - Associated symptoms, medical history/medications
 - Progression of neurological deficits
 - Altered mental status
- Assure an advanced life support response

SYNCOPE/NEAR SYNCOPE

Episode of brief loss of consciousness, dizziness. Often postural, following defecation or early pregnancy. May have cardiac history.

1. Ensure a patent airway (suction as necessary);
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY - Begin oxygen at 6 liters/ minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respirations as needed. **DO NOT** withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Monitor and record vital signs frequently;
5. Assist advanced life support personnel with patient packaging and movement to ambulance.

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PRIORITIES ➤ ABCs ➤ Assess level of consciousness ➤ Evaluate cardiac rhythm, precipitating factors, associated symptoms, medical history and medications; ➤ IV fluids, positioning for hypotension; ➤ Early transport; ➤ EARLY CONTACT OF RECEIVING HOSPITAL.	FINAL DECEMBER 2006
PATIENT TREATMENT GUIDELINES	
➤ Stabilize airway; ➤ OXYGEN THERAPY – high flow as tolerated. Be prepared to support ventilations with appropriate airway adjuncts; ➤ Position of comfort, left lateral decubitus if vomiting; ➤ CARDIAC MONITOR – treat dysrhythmias per specific treatment guideline; rapid transport when able; ➤ IV NS TKO; Complete blood glucose check and treat hypoglycemia (blood sugar = 60 mg/dl). ➤ IV bolus of Normal Saline, 250 cc for BP <90 mmHg systolic or HR >100; if no change in blood pressure or heart rate consider a second bolus of NS 250 ml. ➤ Consider second IV if appropriate..	
DISRUPTED COMMUNICATIONS In the event that a Solano County EMT-P is UNABLE to make physician contact for orders, the paramedic MAY NOT utilize those areas of the protocol needing physician direction and MUST TRANSPORT IMMEDIATELY TO THE CLOSEST FACILITY.	